AM-07 Crashing Medical Patient

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for immediate or impending physiologic collapse requiring simultaneous resuscitation, rapid decisions, and expedited transport.

Immediate-threat and movement pathway

1. Identify	Hypoxia/ventilatory failure, severe distress, SBP below 90/shock, declining mental status, poor perfusion, or peri-arrest dysrhythmia.
2. Assign roles	Leader, airway, monitor/defibrillator, IV/IO/medications, hemorrhage, and documentation. Bring anticipated resuscitation equipment.
3. Immediate threats	Airway/ventilation, severe hemorrhage, tension pneumothorax, hypoglycemia, anaphylaxis, dysrhythmia, and shock.
4. Access + cause care	Rapid IV/IO, reassessed fluids/vasopressors, pads early, and linked definitive protocol.
5. Move + reassess	Move once lifesaving care is underway. Reassess at least every 5 min during prolonged extrication.

Key treatment and disposition

Item	Approved action
Airway	BVM/oxygenation first. Use advanced airway when the patient cannot protect the airway or cannot be adequately oxygenated/ventilated less invasively; confirm with waveform capnography.
Perfusion	Use AM-05 for fluid/vasopressor strategy. Identify peri-intubation hypotension before high-risk intubation when possible.

The goal is not prolonged scene care. Begin immediate lifesaving care at the patient side when safe, then move promptly once critical interventions are underway.

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SAFETY, LINKED PATHWAYS, AND RELEASE CONTROL

Safety checks

- Do not remain on scene when safety is threatened, definitive treatment is unavailable, hemorrhage requires surgery/blood products unavailable to the crew, or transport/rendezvous is the fastest route to needed care.

- GCS 15 does not equal stability. Do not allow procedures to delay definitive care.

- Do not use NIPPV for vomiting, facial trauma, inability to cooperate/protect the airway, or impending arrest.

- Push-dose epinephrine is a temporary bridge only; use the approved labeled preparation/dedicated reference and transition to an infusion when indicated.

Linked pathways

Pathway	Use when
UP-01	Universal patient care
AM-05	Shock/perfusion strategy
AR-01	Adult airway management
AC-03	Cardiac arrest
AM-01	Anaphylaxis
TB-06	Multiple trauma
TE-07	Overdose/toxic ingestion

Documentation and release control

- Document focused findings, BP/rhythm/ECG when applicable, all interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.